Morphine Sulfate

This monograph is under review

Special note: Special care with calculating doses and administration volumes in neonates - frequently involved in medication error, fatal errors have occurred.

CONSIDER DISCUSSION WITH CONSULTANT IN PAEDIATRIC PALLIATIVE MEDICINE FOR PALLIATIVE INDICATIONS OR AT END OF LIFE

FORM **Oral solution 100micrograms/mL**

INDICATION

1. Neonatal Abstinence Syndrome (NAS)
2. Acute Pain
3. Pain in palliative care
4. Dyspnoea in palliative care

DOSE RANGE Neonatal Abstinence Syndrome (NAS)

AGE	DOSE	FREQUENCY	ROUTE
0 – 6 months	Initial dose: 60 micrograms/kg/dose	Every 4 hours	Oral
	If necessary may be increased by increments of 10micrograms/kg/dose to a maximum dose of 80micrograms/kg/dose	Every 4 hours	Oral

Morphine sulphate withdrawal - Morphine sulphate doses should be reduced by decrements of 10micrograms/kg/dose, ideally on a daily basis.

DOSE RANGE – Acute pain

AGE	DOSE	FREQUENCY	ROUTE
0 – 6 months	Initial dose 40 micrograms/kg/dose	Up to every 6 hours	Oral

DOSE RANGE – Pain in Palliative care

AGE	DOSE	FREQUENCY	ROUTE
Neonate	Initial dose 25-50 micrograms/kg/dose	Up to every 6-8 hours Adjust to response (refer to Further Information, point 3 and 4)	Oral/ Enteral
1 – 6 months	Initial dose 50-100 micrograms/kg/dose	Up to every 4 hours Adjust to response (refer to Further Information, point 3 and 4)	Oral/ Enteral

DOSE RANGE – Dyspnoea in palliative care

AGE	DOSE	FREQUENCY	ROUTE
0 – 6 months	Initial dose	As for pain in palliative care	Oral/

Neonatal

	30-50% of palliative pain dose		Enteral

Suggested conversion from continuous IV infusion to oral

10micrograms/kg/hour IV is equivalent to 80micrograms/kg/dose oral every 4 hours

Stop IV infusion after SECOND oral dose

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS
- See Summary of Product Characteristics and most recent edition of BNF for Children

FURTHER INFORMATION

1. Neonates and infants under 1 year show an increased susceptibility to respiratory depression.
2. All non-ventilated babies should be assessed regularly for pain, sedation and respiration.
3. Dose titration in palliative care:
    a. if more than 2 PRN doses given in 24 hours, regular dosing may be required for adequate pain control, seek advice from paediatric supportive and palliative care team (PSPCT)
    b. If regular dosing is prescribed, also prescribe 1/6th to 1/10th of the 24 hour dose as required for breakthrough pain 1-4 hourly (consider discussion with PSPCT if more than 2 PRN doses are given in 24 hours as pain may not be controlled)
    c. Consider discussion with consultant in paediatric palliative medicine for dose titration
4. Consider discussion with consultant in paediatric palliative medicine for pain and dyspnoea at end-of-life as more frequent or regular dosing may be required

STORAGE Morphine sulphate oral solution should be stored as per local protocol.

LICENSED STATUS Unlicensed medicine

APPLICABLE POLICIES West of Scotland Neonatal Guidelines:

Consult local policy if applicable

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Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland pharmacist network. Information is correct at the time of publication and as per local practice agreement.
For further advice contact your clinical pharmacist or pharmacy department.